Oesophagectomy

The main operation used to treat oesophageal cancer is called an oesophagectomy. In this operation, the part of the oesophagus containing the cancer is removed.

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What is an oesophagectomy?

Oesophagectomy is the main operation used to treat oesophageal cancer. In this operation, the part of the oesophagus containing the cancer is removed. The part of the oesophagus that is removed depends on the size and position of the cancer inside the oesophagus.

If the cancer is in the lower part of the oesophagus or has grown into the stomach

The surgeon removes the affected part of the oesophagus and the top of the stomach if the cancer. The surgeon then joins together the remaining parts of the oesophagus and stomach. This forms a tube that becomes the new oesophagus.

If the cancer is in the upper or middle part of the oesophagus.

The surgeon removes this part of the oesophagus. They then pull up the stomach and join it to the remaining part of the oesophagus.

Image: The diagram is a front view of the body. It shows where the remaining part of the oesophagus, has been re-joined to the stomach, after the cancer in oesophagus has been removed. The stomach has been pulled up so it can join the end of the remaining oesophagus, in the middle part of the chest.

You may need to stay in hospital for about 2 weeks after surgery to the oesophagus. Some people may stay longer.

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Your cancer doctor will talk with you about the operation. It is important to discuss the operation with them before it happens. It may help to make a list of questions you want to ask.

Surgery to the lymph nodes

During the oesophagectomy, the surgeon removes a small area of healthy tissue around the oesophagus. This is called a margin. They also remove some of the nearby lymph nodes within the margin. This is called a lymphadenectomy.

A pathologist is a doctor who looks at cells or body tissue under a microscope to diagnose cancer. They look at the lymph nodes under a microscope to check if there are any cancer cells.

Removing the lymph nodes helps reduce the risk of the cancer coming back. It also helps the doctors know more about the stage of the cancer.

Having an oesophagectomy

Depending on where the cancer is in the oesophagus, there are different types of surgery. The 2 main ways a surgeon can do your operation are a trans-thoracic or a trans-hiatal oesophagectomy. But some people may have keyhole surgery.

Trans-thoracic oesophagectomy

The surgeon makes cuts in the tummy (abdomen) and chest to remove the part of the oesophagus that contains the cancer. This is also called a 2 stage oesophagectomy. Sometimes they also make a cut in the neck, which is called a 3 stage oesophagectomy.

Trans-hiatal oesophagectomy

This type of oesophagectomy is not used very often. The surgeon makes cuts in the tummy and neck to remove the part of the oesophagus that contains the cancer.

After surgery to the oesophagus, the stomach will be in a higher position in the body than it was before. It will be above, instead of below, the sheet of muscle (diaphragm) that divides the chest from the tummy. The stomach will also be smaller. This is because the surgeon has shaped it into a tube to replace the part of the oesophagus they removed. This will affect how much you can eat and drink.

Sometimes, it is not possible to join the stomach to the remaining part of the oesophagus. In this case, the surgeon uses a part of the large bowel (colon) to replace the part of the oesophagus they removed. Your cancer doctor will explain this in more detail if they think they might use this type of surgery.

Sometimes during the operation, the surgeon finds that the tumour cannot be removed. This may be because the tumour has spread or gone through the wall of the oesophagus to nearby parts of the body. If this happens, your cancer doctor will talk to you about other treatment options.

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Keyhole surgery (minimally invasive surgery)

The surgeon does this operation through a few small cuts in the tummy (abdomen), rather than 1 large opening. It is sometimes called laparoscopic surgery. The surgeon puts fine tubes with a camera and light on the end through the cuts. This lets them see and work inside the body. Depending on the part of the oesophagus they need to reach, they might work in different areas of the body. A thoracoscopy looks inside the chest, and a laparoscopy looks inside the tummy.

You may be able to have part, or all, of your operation by keyhole surgery. Your surgeon can tell you whether this is suitable for you.

During the operation, the surgeon may decide keyhole surgery is not suitable. They will then do standard surgery instead. Keyhole surgery should only be done by experienced and specially trained surgical teams.

Surgeons sometimes use robotic surgery to treat oesophageal cancer. The surgeon uses controls to move robotic arms that hold the surgical instruments. Having robotic surgery can reduce side effects for some people and may mean they spend less time in hospital.

Possible risks of surgery

Your surgeon will explain the possible risks of oesophagectomy before you have the operation. You will have tests to check your heart and lungs, to make sure you are well enough to have it.

Some of the possible complications of surgery to the oesophagus may be life-threatening. You will usually have a short stay in the intensive care unit after surgery so you can be carefully monitored. Possible risks include:

- a leak in the join between the oesophagus and stomach
- a chest infection or pneumonia
- bleeding
- problems with the wound healing.

It is important to have the information you need about any possible risks before the operation. If you have questions, you can talk to your surgeon or specialist nurse.

After this type of operation, you will be in hospital for a few weeks. At first it may feel like you are improving quite quickly. But it may take a few months before you recover and can go back to your normal activities.

We have more information about having surgery for oesophageal cancer.

Before your operation

Before your operation, you will have tests to make sure you are well enough. These are usually done a few weeks before surgery at a pre-operative assessment clinic. They include tests on the heart and lungs.

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Your surgeon and a specialist nurse will talk to you about the operation. You may meet the doctor who gives you the anaesthetic (anaesthetist) when you are at a clinic. Or you may meet when you go into hospital for the operation.

If you think you might need help when you go home after your operation, tell your specialist nurse as soon as possible. For example, tell them if you might need help because you live alone or are a carer for someone else. Your healthcare team can help organise support before you go home.

You usually go into hospital on the morning of your operation. Or you may go in the night before.

The nurses give you special elastic stockings (TED stockings) to wear during and after the operation. These help prevent blood clots in your legs.

Before surgery, it is important to make sure you have all the information you need. The operation can be complex, so you may want to ask questions. Talk to your surgeon or specialist nurse if there is anything you do not understand.

Enhanced recovery programme

Many hospitals now have enhanced recovery programmes. This aims to reduce your time in hospital and speed up your recovery. It also involves you more in your own care. For example, you will get information about exercises you can do to help you get fitter before surgery. You will also get information about exercises to do after your operation.

It also makes sure any arrangements needed for your return home are organised in advance.

Your cancer doctor will tell you if an enhanced recovery programme is suitable for you.

After your operation

You will probably be cared for in the intensive care unit, or a high-dependency unit, for a few days after your operation. You may be kept asleep for a longer period of time after your operation. Your breathing will be supported using a machine called a ventilator. This is to help your heart and lungs recover after the operation. If you are woken up earlier, a ventilator may be used to help you to breathe for a few hours. You will probably feel quite tired, and may not remember much about the first day or 2 after your operation.

Drips and drains

You may have some drips and drains attached to your body for a few days after surgery. These include the following:

A central venous catheter (CVC or central line)

A CVC is a thin, flexible tube that is put into a large vein in the neck, upper chest or groin. It can stay in place for up to a week. It is used to give you fluids and medicines until you can eat and drink again. It can also be used to take blood samples without a needle.

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A nasogastric tube

A nasogastric tube is a fine tube that goes up the nose and down into the stomach or small intestine. It drains fluid, so you do not feel sick

Chest drains

Chest drains are tubes put into the chest during the operation. They drain away any air or fluid that may have collected around the lungs. The fluid drains into a bottle. If it is uncomfortable, tell your cancer doctor or a nurse.

A feeding tube (jejunostomy)

A feeding tube is a thin, flexible tube that goes into the small bowel through a small cut in the abdomen. It is used to give you food and nutrients until you can eat again.

Abdominal drain

An abdominal drain is a tube that is put into the abdomen to help drain fluid and prevent swelling.

Urinary catheter

A urinary catheter is a tube that is put into the bladder to drain urine into a collecting bag. It can be removed as soon as you can get up and walk around.

Reducing the risk of complications

The nurses will get you out of bed and moving around as soon as possible after the operation. This helps reduce the risk of complications after surgery, such as blood clots and infections. The nurses can help you manage your drips and drains while walking.

A physiotherapist or nurse will teach you deep breathing exercises to help keep your lungs clear. They will also show you how to do regular leg movements to prevent blood clots forming in your legs.

A physiotherapist can also show you how to clear your lungs of any fluid that may have built up because of your operation.

Pain

You will probably have some pain and discomfort after the operation. Your cancer doctor or specialist nurse will explain how your pain will be controlled.

You may have painkillers in one of the following ways:

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Into the space around the spinal cord (epidural)

The painkillers are given through a very fine tube that the surgeon places into your back during surgery. The tube connects to a pump, which gives you a continuous dose of painkillers.

Patient-controlled analgesia (PCA)

A painkiller is given through a pump that you control. This allows you to give yourself an extra dose of pain relief when you need it.

When you no longer need the epidural or PCA, you have painkillers as tablets or liquids.

It is important to tell your healthcare team if you are still in pain. Mild discomfort or pain in your chest can last for several weeks, so they will give you some painkillers to take home with you.

Your wound

You will probably have a dressing covering your wound, which might not be removed for the first few days.

How long the wound takes to heal depends on the operation you had. The surgeon may have closed your wound with glue or stitches that dissolve and do not need to be removed. If you do not have stitches that dissolve, they are usually removed about 7 to 10 days after your operation.

Tell a nurse or your cancer doctor straight away if your wound becomes:

- hot
- painful
- leaks any fluid.

Eating and drinking after surgery

You will not usually have anything to eat for the first 48 hours after surgery. When you are fully awake, you may have small sips of clear fluids. The amount of fluids you have is slowly increased. After a few days, when you can drink enough, you will start having small amounts of soft foods, and then normal food in smaller portions. This means the new joins that were made during surgery have some time to heal. In some hospitals you may have tests to check there are no leaks in the new join. This might be a CT scan.

Feeding tube

You will usually go home with your feeding tube still in, to make sure you get enough food and nutrients and do not lose weight. Before leaving hospital, your nurse or dietitian will show you how to use your feeding tube. If you have a carer, they can learn how to use it too. You will meet the dietitian regularly as an outpatient to check how well you are eating. When you are eating and drinking enough, the tube can be removed.

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Recovering after your operation

You will still be recovering for some time after you go home. Try slowly building up the amount of physical activity you do. Gentle exercise, such as regular walks, builds up your energy levels. You can increase the amount you do as you feel better. Everyone is different, and some people take longer than others to recover.

Avoid lifting heavy things like shopping, or doing vacuuming or gardening, for at least 8 weeks. This gives your wound time to heal.

Some insurance policies give specific time limits for not driving after surgery. Contact your insurance company to tell them you have had an operation. Most people are ready to drive about 4 to 6 weeks after their operation. Do not drive unless you feel in full control of the car.

We have more information about going home from hospital, which explains how to get support after you get home.

Outpatient appointment

Before you leave hospital, you will be given an appointment for your check-up at an outpatient clinic. You may also be given contact details for your specialist nurse. The appointment is a good time to talk about any problems you have after your operation. If you have any problems before this appointment, you can contact your cancer doctor, specialist nurse or ward nurse for advice.

Eating

You probably will not feel like eating very much for a while, and you may lose some weight. You may feel full after eating small amounts. Some foods might make you feel sick, or give you indigestion or diarrhoea. It is important to try to eat, even if you do not feel like it. Over time, you will start to manage bigger portion sizes and different foods. If you continue to have problems, it can help to write down what is happening so you can limit or avoid certain foods. If you continue to have problems with food, talk to your cancer doctor or dietitian.

We have more information about eating after treatment for oesophageal cancer.

Booklets and resources

Booklets

01 Apr 2025

Eating problems and cancer

Indigestion

Indigestion or reflux is a backward flow of digestive juices into the oesophagus. You may have it after oesophagus surgery. This can cause pain and inflammation of the lining of the oesophagus. Your GP or cancer doctor can prescribe antacid medicines to help with this.

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Avoid fizzy drinks, alcohol and spicy foods, as these may make your symptoms worse. Lying down may also make symptoms worse. After eating and drinking, try to stay sitting up for at least an hour. When you lie down to sleep, try using pillows to raise your head slightly.

Indigestion can also be caused by wind trapped in the digestive system. You can reduce wind by drinking peppermint water. You can buy this from a pharmacy.

Preventing weight loss

To start with, you may find it hard not to lose weight. Your body will use lots of calories to help it recover from surgery and you may not be eating as much as you normally would. But in time, most people find they stop losing weight.

You will usually meet a dietitian, who will talk with you about the effects of surgery on your diet. They will give you advice about eating a balanced diet and building up your weight. To gain weight, you need to add more energy (calories) and protein to your diet. You can do this by:

- eating high-calorie foods, such as crisps, cakes, biscuits and pastries
- adding more calories to your food by using things like cream, butter or cheese
- having nutritious, high-calorie and high-protein food supplements – these are available on prescription as liquids or powders.

Dumping syndrome

The stomach normally stores food and releases it into the bowel in a controlled way. After an operation to remove part of the oesophagus, food can travel more quickly through the digestive system. This can cause symptoms called dumping syndrome.

We have more information about nutritional support and diet problems after surgery.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References

Below is a sample of the sources used in our oesophageal cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

European Society for Medical Oncology. Oesophageal Cancer: ESMO clinical practice guidelines for diagnosis, treatment and follow up. 2022. Available from www.esmo.org/guidelines/guidelines-by-topic/gastrointestinal-cancers/oesophageal-cancer [accessed July 2023].

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Our cancer information meets the PIF TICK quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about how we produce our information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as ‘you’ so that everyone feels included. Where clinically necessary we use the terms ‘men’ and ‘women’ or ‘male’ and ‘female’. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information here.

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How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.

Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.

Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don’t want to call.

Oesophageal cancer forum

An anonymous network of people affected by oesophageal cancer which is free to join. Share experiences, ask questions and talk to people who understand.

Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help

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